



Advocacy at the Center. Partnership as the Path.

EmpathIQ Advisors Website design review

A five-page site built from the brand core materials, the founder bio, and the advisory network profiles. Prepared for Lee Lynch.

Design system

Colors are sampled directly from the logo artwork rather than chosen. The wine tone is the smallest amount of ink in the mark and sits at its dead center, so the site uses it only at focal points: the active nav item, link underlines, and the "without early advocacy" state.



Playfair Display

Headlines and the italic tagline. Chosen because it closely matches the high-contrast serif already in the wordmark.

Public Sans

Body and interface text. It is the typeface of the U.S. Web Design System, which suits a practice working in federal health policy.

Reference sites

Aery gave the four-page shape and the speaking-engagements section. **JPA** gave the "who we help" sector row and the flexing-bench framing. **Fenton** gave the "why clients choose us" block and testimonial treatment. **G2G** gave the proof strip under the hero.



SKIP



SKIP



SKIP

Advocacy at the Center. Partnership as the Path.

A US-based global consultancy working at the intersection of health and social issue advocacy, communications, policy, and research-powered insight.

When a cause needs a voice, advocacy is the lever.

[Talk with us →](#)

[See what we do](#)

25+ years

of founder-led advocacy and coalition-building.

A networked bench

assembled around each engagement.

US-based, global

across disease areas and health systems.

THE CASE FOR EARLY ADVOCACY

Early advocacy is not a moral aspiration. It is a strategic imperative.

Every organization developing a product, pursuing policy change, or advancing a cause is ultimately trying to build a relationship with a person.

The question is not whether to build that relationship, but whether to start now, when it shapes everything that follows, or later, when it is harder and far more likely to fail.

What changes at each stage, depending on when advocacy starts

Without early advocacy With early advocacy

01	Pre-clinical & discovery	The community is unknown. Trial design reflects internal assumptions rather than lived experience.
02	Phase 1 / first-in-human	No channel to the patient community. Sites are chosen by default, and relationship-building becomes a late scramble.
03	Phase 2–3 / pivotal trials	Enrollment gaps, distrust of the sponsor, diversity shortfalls, protocol amendments, timeline slippage.
04	Regulatory submission	Patient voice is added late and advocacy letters are assembled reactively, leaving the dossier weak on real-world burden.
05	Access & commercialization	Payer resistance, community skepticism, and limited uptake in the communities that need the therapy most.

Outcomes depend on program-specific factors. We're glad to share illustrative examples and to define success metrics together for a specific engagement.

01 / Services

How we help clients

Five areas, anchored in advocacy and amplified by partnership.

See all five in detail →

Advocacy & alliance building	Coalitions, campaigns, and patient engagement across disease areas and policy landscapes.
Communications & campaign strategy	Evidence-based, empathy-driven, and built around partnership rather than a single voice.
Access, policy & regulatory	Approvals, lifecycle strategy, access expansion, and value demonstration.
Capacity building	Infrastructure, capabilities, and partnerships that let organizations scale.
Stakeholder & landscape research	Research and analytics that turn stakeholder insight into decisions.

02 / Clients

Our clients

- | | |
|---|---|
| <ul style="list-style-type: none">Biopharma & life sciencesPatient advocacy organizationsNonprofits & foundations | <ul style="list-style-type: none">Medical & professional societiesPublic health & governmentHealth technology |
|---|---|

03 / Network

We assemble the team the challenge demands

Senior advisors sourced around the specific problem, flexing as the engagement evolves.

[Explore the advisory network →](#)

- Advocacy & patient engagement
- Policy
- Clinical perspective
- Campaigns
- Corporate affairs
- Media
- State advocacy
- Global breadth
- Research
- Digital & creative
- Writing & video
- Artificial intelligence



04 / LEADERSHIP

Lee Lynch

CEO and Founding Partner

Twenty-five years building advocacy functions, coalitions, and campaigns across health and social issue work.

[Read more about Lee and the practice →](#)

Tell us what's standing in the way.

A regulatory hurdle, a policy environment that hasn't moved, a community whose trust hasn't been earned, a coalition that hasn't come together. Advocacy is very often the key, and the first conversation costs nothing.

[Talk with us →](#)

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- 571-214-8799
- LinkedIn

OFFICE

- Alexandria, Virginia
- US-based, global reach

PRACTICE

- Advocacy & alliance building
- Strategic communications
- Access & policy

WHO WE ARE

Advocacy, partnership, and communications — and the judgment to know which the moment calls for.

A US-based global consultancy built on a conviction shaped by every campaign, coalition, and cause we have touched.

That conviction: advocacy — principled, strategic, and deeply human — is the most powerful force available to companies and organizations that want to bring about change for patients and people.

Three disciplines sit at the center of the work: advocacy, partnership, and strategic communications. Each is a genuine strength on its own, and we know when to pursue them separately and when to pursue them together.

When a company cannot move a market, when a policy stands between a patient and the treatment they need, when a clinical trial cannot find the community it is meant to serve, when a cause finds itself without the voice or the allies to carry it forward, advocacy is the lever.

We are structured as a networked advisory practice, built to assemble the right people, voices, and strategies around any challenge. Time and again we have seen that organizations unable to achieve what they want — therapies reaching fewer patients than they should, coalitions whose voices go unheard, causes that stall at the edge of possible — are missing a better-coordinated advocacy strategy rather than a better product.

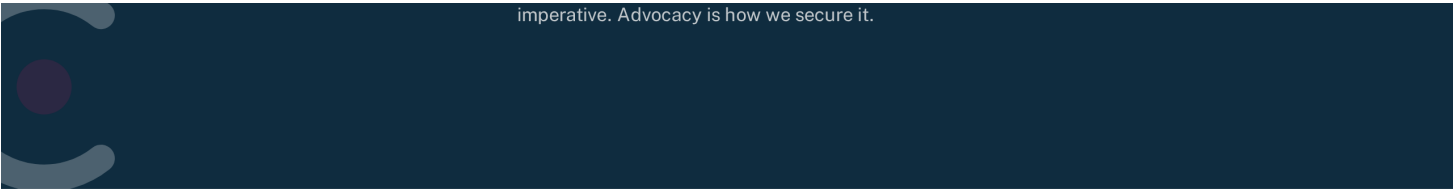
So we ask not just what a client needs, but who else needs to be at the table, what others need to hear from them, and how to make sure the right people are listening.

Advocacy and alliance-building are rarely one-time events. They are ongoing disciplines, which is why a substantial share of our client relationships evolve into sustained, retained partnerships. We are built, staffed, and resourced to be that ongoing partner.

01 / MISSION

To advance patient outcomes and social progress by building the advocacy infrastructure that transforms human insight into intelligent action.

The partnerships, campaigns, coalitions, and strategies. Access to knowledge, treatment, care, and the systems that govern them is



02 / Difference

What sets us apart

Eight things clients tell us they do not find elsewhere.

Deep credibility	25+ years of founder expertise across women's health, aging, rare disease, oncology, neuromuscular and genetic disorders, chronic care, sexual health, infectious disease, vaccines, and global health equity.
Networked expertise	We source and coordinate best-in-class subject matter experts, advocates, and specialists for each engagement, giving clients exactly the team their challenge demands.
Empathy and intelligence	Emotional intelligence paired with analytical rigor, integrating responsible AI to amplify human judgment rather than replace it.
Lean and accountable	Senior leadership engages directly on accounts, and the networked model flexes capacity up or down, including multi-region, multi-workstream programs.
Partnership	Shared goals, transparent communication, and outcomes measured by what changes for people. We bring compliance fluency so partnerships are built right from the start and hold up under scrutiny — transparency requirements, anti-kickback considerations, conflict-of-interest disclosure — working alongside client compliance and legal teams.
Mission-aligned	Every engagement is passionately pursued for its potential to advance patient and social outcomes, and we measure success by what changes for people.
Built for ongoing partnership	Many engagements begin as a defined project and evolve into a retained relationship: sustained advocacy counsel, alliance management, and communications support as a client's needs change.
Communications built around partnership	The most effective communications often aren't built around a single voice. They're built around credible voices aligned on one message, changing minds and shaping behavior further than any organization can alone.

03 / MINDSET

Our partnership mindset

Advocacy does not happen alone. It happens when the right voices find one another, when shared purpose becomes shared strategy, and when individual commitment scales into collective power.

Every partnership we engage in — whether with a global advocacy organization or a single patient advocate — reflects genuine respect for that partner's expertise, experience, and humanity. Partnerships are designed to create value for all parties. Trust is built over time, through consistent follow-through, honest communication, and shared commitment to mission.

This is also why so many EmpathIQ relationships last for years. Partnership is not a one-time achievement. It is a sustained practice, and we are built to be the sustained partner for it.



04 / LEADERSHIP

Lee Lynch

CEO and Founding Partner

Lee has a passion for turning collaboration into real-world change, amplifying human insight through responsible technology to advance outcomes and causes. She has led advocacy and alliance development functions and advocacy expert advisory boards at numerous consulting firms, and has helped build and manage many initiatives, campaigns, and organizations, with deep expertise in writing, strategy, and execution across advocacy and communications.

She founded EmpathIQ Advisors, formerly Lynch Advocacy Solutions, on the conviction that advocacy done early and done well is the most powerful force available to organizations trying to change something for patients and people.

Her support for neurodiverse individuals and causes led her to found Rocket Around, a content provider supporting adventure and life for neurodiverse families through its website, social channels, and books including *Rocket Around Washington DC*, *Rocket Around Switzerland*, and *Rocket Around Germany*. Lee is a travel writer, a trained professional moderator, and an executive board member of the Culmore Clinic, a nonprofit offering free, compassionate medical care, counseling, and specialty referrals to uninsured adults in northern Virginia. Lee and her family live in Alexandria, VA.

- Master's degree in journalism, University of South Carolina
- Bachelor's degree in speech communications and mass broadcasting, UNC Chapel Hill
- Autism Certificate, International Board of Credentialing and Continuing Education Services
- Trained professional moderator, panels, advisory boards, and stakeholder convenings

Connect with Lee on LinkedIn →

Tell us what's standing in the way.

A regulatory hurdle, a policy environment that hasn't moved, a community whose trust hasn't been earned, a coalition that hasn't come together. Advocacy is very often the key, and the first conversation costs nothing.

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PRACTICE

- Advocacy & alliance building
- Strategic communications
- Access & policy

WHAT WE DO

Five service areas, anchored in advocacy.

If something stands between your organization and the outcome you are working toward, advocacy and engagement are very often the key.

Clients

Our clients

- | | |
|----------------------------------|------------------------------------|
| • Biopharma & life sciences | • Medical & professional societies |
| • Patient advocacy organizations | • Public health & government |
| • Nonprofits & foundations | • Health technology |
| • Coalitions & alliances | • Social issue campaigns |

01

Advocacy & alliance building

We design and manage coalitions, advocacy campaigns, and patient, consumer, and population engagement initiatives across disease areas and policy landscapes. A single patient's story can move a room. A thousand patient stories, coordinated and directed with purpose, can move a legislature, a regulator, or a market.

We create the conditions for that kind of collective power through stakeholder and influencer mapping, partner identification, coalition governance, and the convening strategies that turn individual voices into undeniable collective impact. Because coalitions require continuous tending rather than a one-time build, much of this work is ongoing advocacy counsel and alliance management rather than a single campaign.

02

Communications & campaign strategy

We develop evidence-based, empathy-driven communications strategies for complex health and social challenges: scenario planning and issues management, mis- and disinformation prediction, pre-emption and response, message development and testing, and media engagement. We help clients say the most impactful things, to the right people, at the right moment.

We see communications differently, with partnership at the center rather than the periphery. When credible voices carry one message together, it changes minds and moves behavior further than any single organization speaking alone, from coalition messaging to multi-sponsor campaigns to alliance communications that hold together under real organizational tension. We bring the same rigor to focused, single-organization work.

03

Access, policy & regulatory communications

We help advance drug and product approvals, product lifecycle strategies, access expansion programs, and value demonstration campaigns, with deep experience supporting regulatory submissions through patient and advocacy engagement and navigating the policy landscapes that shape what patients can access and when.

In our experience, regulators respond well to organized, credible, sustained patient engagement and poorly to last-minute advocacy letters. We help build that record from the beginning, working alongside your regulatory affairs and legal teams. We provide advocacy and communications strategy, not regulatory or legal advice.

04

Capacity building & organizational development

We help advocacy organizations, patient groups, and companies build the infrastructure, capabilities, and partnerships they need to scale their impact: organizational creation and development, bandwidth expansion, operational strategy, strategic planning, communications strategy and execution, and leadership in specific competency areas and markets.

05

Stakeholder & landscape research and analytics

We apply primary and secondary research to gather and organize stakeholder, influencer, and situational insight that helps clients build strategy and make key decisions. Decades of creating stakeholder and landscape research processes for major communications firms — and acting on the findings — shape how we approach it.

Where warranted, we use responsible AI and data analytics to surface patient insight, identify advocacy opportunities, monitor emerging issues, and measure campaign outcomes with precision. AI amplifies human expertise; it does not replace it. Critically, it gives smaller companies the intelligence infrastructure of a much larger one. We use only public or client-licensed, de-identified data sources, follow applicable data privacy regulations, and disclose our AI tools and data handling practices on request.

HOW WE WORK

Five principles, in this order.

01

Listen first

We begin with genuine empathy, understanding the human stakes before designing any strategy: first-hand stakeholder listening, patient and community engagement, an honest assessment of what is and isn't working, and online listening to gather a broader set of perspectives.

02

Map the ecosystem

Every health and social challenge exists within a web of partners, stakeholders, and influencers. We identify who needs to be at the table, what

they care about, and how to connect them around a shared purpose.

03	Build the right team	We draw on our global network of subject matter experts, advocacy leaders, communications specialists, and technology partners to assemble precisely the expertise each engagement requires.
04	Design with evidence	Our strategies are evidence-based and technology-enhanced. We supplement primary data with research and analytics to surface insight, test messages, identify opportunities, and measure what matters.
05	Execute with accountability	We measure outcomes that matter to people and to clients, with clear milestones, transparent communication, and a commitment to reporting.

What changes at each stage, depending on when advocacy starts

		<u>Without early advocacy</u>	With early advocacy
01	Pre-clinical & discovery	The community is unknown. Trial design reflects internal assumptions rather than lived experience.	
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Outcomes depend on program-specific factors. We're glad to share illustrative examples and to define success metrics together for a specific engagement.

Tell us what's standing in the way.

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OUR NETWORK

We assemble the team the challenge demands.

Senior advisors across advocacy, policy, clinical practice, media, research, creative, and technology. The team flexes as an engagement evolves.

ADVOCACY & POLICY

01

Advocacy & patient engagement

- Health equity & women's health founders
- Nonprofit & philanthropic strategy
- Pharmaceutical public affairs
- Social justice organizing
- Patient engagement pioneers
- Cross-aisle legislative strategy
- Alliance development
- Reimbursement & policy

02

Policy

- FDA regulatory consulting
- Medicaid, health IT & managed care
- Political strategy, both parties
- MedTech, imaging, diagnostics & AI
- Former FDA official, innovation think tank
- Market access & drug pricing
- Federal & state maternal and child health
- Financial markets

03

State advocacy

- Award-winning strategists
- Multi-state firm reach
- Published authors
- Coordinated advocacy & media campaigns

04

Global breadth

- Advisors across countries and continents
- Approval & reform
- Policy & communications
- Multiple health and coverage systems
- Patient engagement

05

Clinical perspective

- Vaccines & infectious disease
- Cardiology
- Patient safety
- Obstetrics, gynecology & menopause
- Neuromuscular & rare disorders
- Medical society leadership
- Public health & misinformation
- Health equity
- Pharmacy practice & care coordination
- Mental, behavioral & developmental health
- Aging & chronic conditions

06

Research

- General population polling
- Investment-sector analysis
- Evidence-based science communication
- Market research
- Message testing
- Media & data analytics
- HCP & specialist research
- Clinical trial expertise

COMMUNICATIONS

07

Campaigns

- Advocacy
- Public relations
- Public affairs
- Digital

08

Corporate affairs
& account strategy

- Life sciences narrative strategy
- Consulting firm founders
- National medical society leadership

09

Media

- Independent PR consulting
- US & global agency leadership
- Reputation under pressure
- Washington communications strategy
- Newsroom relationships

10

Writing & video

- Strategic storytelling in any medium
- Film, live event & digital content
- Script-to-screen studio

CREATIVE & TECHNOLOGY

11

Digital & creative

- In-house creative direction
- Independent designers & illustrators
- Full-service branding agency
- Marketing & design
- Brand & web strategy

12

Artificial intelligence

- Safety-first AI engineering
- AI strategy & published authorship
- Human oversight by design
- Turning capability into measurable growth

Also

Additional advisors
and partners

- Management consulting
- Faith-based engagement
- Employer & benefits policy
- Academic thought leadership
- Health economics & outcomes
- Technology commercialization
- Fundraising & development
- Multi-channel marketing

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Home	lee@empathiqadvisors.com	Alexandria, Virginia	Advocacy & alliance building
About	571-214-8799	US-based, global reach	Strategic communications
What we do	LinkedIn		Access & policy
Our network			
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GET IN TOUCH

Tell us what's standing in the way.

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Direct

Reach us

Lee reads every inquiry personally and usually replies within two business days.

• lee@empathiqadvisors.com

• 571-214-8799 (cell)

• [LinkedIn](#)

• Alexandria, Virginia

YOUR NAME

ORGANIZATION

EMAIL

WHAT BRINGS YOU HERE

Advocacy & alliance building

WHAT'S STANDING IN THE WAY?

Send message →

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- Patient advocacy organizations
- Nonprofits & foundations
- Coalitions & alliances
- Medical & professional societies
- Public health & government



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